

OET WRITING

COMPREHENSIVE RULEBOOK

Sessions 1–10 · All Letter Types · Grammar & Criteria

THE TUTOR BOOK · OET Preparation by Dr. Ahmed Hesham · @DrAhmedHesham

HOW TO USE THIS RULEBOOK

Every OET Writing rule from Sessions 1–10 is organised into 16 topic sections. Rule codes: R03.2 = Section 3, Rule 2. CRITICAL rules cause automatic mark deductions if violated. Templates, format patterns, and exceptions are all included.

01 LETTER TYPES & IDENTIFICATION

R01.1	Six OET letter types: Routine Referral, Urgent Referral, Discharge Letter, Transfer Letter, Referral to Non-Medical Professional, and Referral to GP.
R01.2	Any referral task without the word "urgent" = Routine Referral by default.
R01.3	Case notes requesting "assessment" or "management from a specialist" without "urgent" = Routine Referral.
R01.4	Urgent identifiers: "urgent referral/assessment", "admit to hospital", "hospital admission", "request acute management", "as soon as possible (ASAP)", "acutely unwell", "for immediate assessment".
R01.5	CRITICAL — Suspected cancer = ALWAYS apply Urgent Referral rules, even if the word "urgent" is absent.
R01.6	Discharge letter identifiers: "write a discharge letter", patient described as in hospital or being discharged, you are writing from hospital to GP.
R01.7	Non-medical professionals: OT, physiotherapist, social worker, psychologist, dietitian, speech pathologist, pharmacist, care facility manager. Nurse is classified as medical, not non-medical.
R01.8	Do not confuse routine cancer screening referrals with urgent referrals for suspected cancer.

02 EXAM STRUCTURE & CASE NOTES

R02.1	Total exam time: 45 minutes.
R02.2	CRITICAL — First 5 minutes = reading time only. No writing is permitted; pen must remain down.
R02.3	Remaining 40 minutes = writing time. Practice target: finish in 30 minutes, leaving 10 minutes for review.
R02.4	Paper 1 = case notes (usually 2 pages). Paper 2 / Booklet = writing answer sheet.
R02.5	The written task (bottom of case notes) states: letter type, recipient name + address, request, and sometimes the diagnosis.
R02.6	If diagnosis is absent from the written task, check the "plan" section immediately above it.
R02.7	"Today's date" at the top of case notes = the date of the letter.
R02.8	Diagnosis types — provisional, preliminary, evaluatory, confirmatory — are treated identically in OET letters.

03 CONTENT & DATA SELECTION

R03.1	All data falls into one of three categories: Relevant (must include), Irrelevant (must exclude), Semi-relevant (include if letter is short; exclude if long).
R03.2	CRITICAL — Marks are awarded for INCLUDING relevant data AND for EXCLUDING irrelevant data. Content selection is the single most important OET writing skill.
R03.3	The recipient specialty determines relevance. Ask: "Would this specialist need this to manage the patient?"
R03.4	CRITICAL — Smoking and drinking: ALWAYS include (positive or negative), EXCEPT when writing to an occupational therapist.
R03.5	Divorce, living situation, number of children: include only if directly relevant to the specialty.
R03.6	CRITICAL — Allergy: ALWAYS include (positive or negative) for atopic conditions (asthma, eczema, hay fever). For non-atopic conditions, include allergy only if relevant to the condition or its medication.
R03.7	Family history: include only if a family member has a related condition.
R03.8	Letter length guide: 180–200 words (body only). Too long = semi-relevant data included. Too short = relevant data missing.

R03.9

Maximum 4 body paragraphs. Typical: 2–3 body paragraphs. Paper-based: fills to approx two-thirds of second page.

04 PAGE LAYOUT & STRUCTURE

R04.1

Letter structure order: Address → [blank line] → Date → [blank line] → Salutation → Re: line → Introduction → [blank line] → Body paragraphs (each separated by blank line) → [blank line] → Closure → [blank line] → Yours sincerely/faithfully → [1–2 blank lines] → Doctor.

R04.2

CRITICAL — NO blank line between the Salutation and the Re: line. They are on consecutive lines.

■ *Salutation and Re: line are the ONLY two consecutive lines with no blank line between them.*

R04.3

Computer-based exam: two Enter presses = one blank line. "Space" or "blank line" throughout this rulebook means 1–2 Enters on the keyboard.

R04.4

Each body paragraph is separated from the next by exactly one blank line.

05 ADDRESS & DATE

- R05.1** Address components: recipient name, specialty, clinic name, street, city/town. Number of lines varies; not always five.
- R05.2** **CRITICAL — Address: first letter of each component = CAPITAL. No commas or full stops. Use Enter (not comma) between components.**
- R05.3** Address is always copied directly from the written task in the case notes.
- R05.4** Date of the letter = date of today's visit (at top of case notes).
- R05.5** Date format must be consistent throughout the letter. Acceptable formats: DD/MM/YYYY, 1 January 2024, 01/01/2024, 1st January 2024.
- R05.6** Never abbreviate the year (write 2024, not '24).
- R05.7** Numbers 1–9 in dates: always add a leading zero (01, 02 ... 09).
- R05.8** **CRITICAL — Do NOT write the word "Date:" before the date.**
- R05.9** The date has exactly ONE blank line above it and ONE blank line below it.
- ONE blank line above date → date → ONE blank line below date → Salutation.

06 SALUTATION & RE: LINE

- R06.1** **CRITICAL — Standard: Dear Dr. [Last Name], — capital D, lowercase r. Always use LAST name, never first name.**
- R06.2** Comma after salutation: optional. Dot after Dr/Mr/Ms: optional.
- R06.3** **CRITICAL — No blank line between salutation and Re: line.**
- R06.4** Special salutations: No name given → Dear Doctor. Nurse/OT/Radiographer without name → Dear Sir/Madam. Admissions Officer → Dear Admissions Officer. Emergency Registrar → Dear Emergency Registrar. Assistant Professor → Dear Assistant Professor. UK surgeon → "Mr [Last Name]" acceptable.
- R06.5** Non-medical professional — no name: Dear Sir/Madam. Name given: Dear Mr/Ms [Last Name].
- R06.6** Re: line format: Re: [Title] [Full Name] D.O.B: [date]. Both "Re:" and "RE:" are acceptable.
- R06.7** **CRITICAL — Re: line = full name (first + last). In body: LAST NAME + TITLE only (e.g., Ms Miller). Never repeat full name in body.**
- R06.8** When only DOB is given → write DOB in Re: line. When only age is given → write "aged [X]" (lowercase, no colon) after the full name. Do NOT write the age again in the introduction. When both are given → use DOB in Re: line; age is optional in the introduction.
- "Re: Mr John Smith, aged 55" — lowercase 'aged', no colon, no "Age:" prefix.

Situation	Re: Line Format	In Introduction
DOB given	Re: Ms Sara Miller D.O.B: 01/01/1984	Age optional in intro
Age only given	Re: Ms Sara Miller, aged 40	Do NOT repeat age in intro
Both given	Re: Ms Sara Miller D.O.B: 01/01/1984	Use DOB in header; age optional

- R06.9** Age in running text (three acceptable forms): "Age: 25" / "25 years old" / "25 years of age".

R06.10 **CRITICAL — Children (Day 1 of life up to and including 17 years old): NO title in Re: line — write full name only. In the body: use FIRST NAME only. At 18 years old the patient becomes an adult and receives a title (Mr/Ms/Miss).**

■ A 17-year-old = child (no title). An 18-year-old = adult (title required). Never write "Master" for boys.

■ **Never write "Master" for boys — just first name in Re: line for all children.**

Rule	Children (Day 1 – 17 years)	Adults (18 years+)
Re: line	Full name only — no title Re: Sara Miller D.O.B: 01/01/2015	Title + full name Re: Ms Sara Miller D.O.B: 01/01/1999
Body — first mention	First name only: "Sara presented with..."	Title + last name: "Ms Miller presented with..."
Body — same paragraph	Pronoun: "she reported..." / "her symptoms..."	Pronoun: "she reported..." / "her symptoms..."

R06.11 **CRITICAL — "Yours sincerely" when recipient name is known. "Yours faithfully" when no name (Dear Sir/Madam, Dear Doctor).**

R06.12 Capitalisation: Yours (capital Y), sincerely (lowercase s). Spelling: S-I-N-C-E-R-E-L-Y.

R06.13 Write "Doctor" (capital D) below closing phrase. Allow 1–2 blank lines between closing phrase and "Doctor".

07 INTRODUCTION

R07.1 Introduction must be concise: 2–3 sentences maximum, fewest words possible.

R07.2 Introduction must contain: (a) Unique Identity of the patient, and (b) Purpose / Request.

R07.3 **CRITICAL — Purpose/Request = MOST IMPORTANT element. Never omit it.**

R07.4 Unique identity elements: name, age (if not in header), occupation, summary of main condition.

R07.5 Routine referral template: "I am writing to refer [Title Last Name], a [age]-year-old [occupation], for [specialist] assessment and management of [condition]."

R07.6 **CRITICAL — Urgent referral: MUST contain the word "urgent" — "I am writing to urgently refer..." / "I am writing to request urgent assessment of..."**

R07.7 **CRITICAL — Discharge letter to GP: Do NOT describe patient identity in introduction — GP knows the patient.**

R07.8 Discharge introduction template: "I am writing to update you regarding Mr Jones, who underwent [procedure] at [hospital] and is now ready for discharge, for whom I am requesting your ongoing care."

R07.9 Appositive clause: "Ms Miller, a 25-year-old teacher, presented with..." — use "gentleman" for males, "lady" for females (not man/woman).

08 BODY PARAGRAPHS & VISIT STRUCTURE

R08.1 **CRITICAL — Minimum 2 body paragraphs. NEVER write the entire letter in one paragraph.**

R08.2 Today's visit must always have its own paragraph. Exception: only one item of information today → may combine with adjacent visit.

R08.3 **CRITICAL — Previous visits may be combined or summarised. Do NOT give every visit its own paragraph.**

R08.4 Visit structure: Subjective (symptoms) + Objective (examination) + Management.

R08.5 **CRITICAL — Urgent referral: ALWAYS begin body with TODAY'S VISIT (most acute presentation), then background chronologically.**

R08.6 Visit referencing: prefer elapsed time over exact dates — "3 months ago" preferred over the exact date.

R08.7 **CRITICAL — Next visit reference: "on the following visit" / "later on" / "later that month". NEVER "next visit".**

R08.8 **CRITICAL — NEVER write today's date in the body. Use "On today's visit" or "On today's presentation" instead.**

R08.9 "Yesterday" never used in a medical letter. Write "one day prior" or "one day ago" instead.

R08.10 "Over the last [X time]": summarises multiple visits with the same complaint. Requires present perfect. Use only to START a paragraph.

R08.11 Duration as adjective (before noun): hyphenate, no plural: "a 6-month history of acne". Duration standalone: plural: "For 6 months, she has had acne."

R08.12 Condition unchanged: "persisted" / "remained unchanged". Improved: "there was partial improvement". Worsened: "deteriorated" / "worsened".

R08.13 Advice: "She was advised to adopt lifestyle changes." / "She was counselled regarding..." (never "instructed to adopt").

R08.14 **CRITICAL — In body: last name + title only (Ms Miller). Never use full name again. Never use "the patient".**

R08.15 Name vs. Pronoun within paragraphs: Use the patient's name (title + last name for adults; first name for children) at the FIRST mention in each paragraph. For ALL subsequent mentions within the SAME paragraph, use a pronoun (he/she/him/her/his/her). At the start of EVERY new paragraph, use the name again.

■■ NEVER write "the patient" anywhere in the body. Name at the start of each paragraph, pronoun for all further references within that paragraph.

Mention	Adults	Children
First mention in each paragraph	Title + Last name "Ms Miller presented with..."	First name only "Sara presented with..."
Further mentions — same paragraph	Pronoun "She reported..." / "Her temperature..."	Pronoun "She reported..." / "Her temperature..."
First mention — next paragraph	Title + Last name again "Ms Miller returned one month later..."	First name again "Sara returned one month later..."

Example: "Ms Miller presented with a 3-week history of severe headaches. She reported associated nausea. Her observations were normal. [NEW PARA] Ms Miller returned on the following visit reporting improvement. She was continued on treatment."

09 CLOSURE

R09.1 Standard phrases: "I trust this information will be of use." / "Please do not hesitate to contact me should you require further information."

R09.2 **CRITICAL — Urgent closure MUST include "at your earliest convenience." Spelling: C-O-N-V-E-N-I-E-N-C-E.**

- R09.3** Use "urgent" once in introduction only; use "at your earliest convenience" in closure to imply urgency — do not repeat "urgent" throughout.
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- R09.4** **CRITICAL — Follow-up / review date stated in case notes → ALWAYS mention in closure.**
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- R09.5** **CRITICAL — Copy of results/imaging stated as enclosed → write "Please find enclosed a copy of Mr [Name]'s pathology/imaging results." Do NOT also list the values in the letter.**
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- R09.6** Copy of medication list attached → list medication names only (no doses).
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- R09.7** Patient-initiated referral → "upon his/her request" or "at Mr Jones' request" in introduction or closure.
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- R09.8** Consent in case notes → "Please note that Mr Jones has been fully informed of his diagnosis and management."
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- R09.9** Always leave ONE blank line before "Yours sincerely," / "Yours faithfully,". The last body paragraph and the closing phrase must be separated by a blank line.
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■ "...I would be grateful if you could review Mr Smith. [BLANK LINE] Yours sincerely,"

10 TENSES

R10.1 Introduction: present simple or present perfect (both acceptable).

R10.2 **CRITICAL — All visits (today and previous): PAST SIMPLE. Safest choice; never wrong for visit content.**

R10.3 Present perfect for a visit: acceptable only if the symptom is still present at time of writing. CANNOT use present perfect for vital signs.

R10.4 Medical/social/family history — ongoing facts: present simple. "She has hypertension."

R10.5 **CRITICAL — "Since" + starting point → present perfect: "She has had hypertension since 2010."**

R10.6 **CRITICAL — "For" + duration → present perfect: "She has had hypertension for 5 years."**

R10.7 Smoking/drinking with duration → present perfect continuous: "She has been smoking for 20 years."
Without duration → present simple: "She smokes."

R10.8 **CRITICAL — Resolved conditions and surgery → PAST SIMPLE. "She had a cholecystectomy in 2018." Never use present perfect for surgery.**

R10.9 "Over the last [X]": first sentence → present perfect; remaining sentences in same paragraph → past simple acceptable.

R10.10 **CRITICAL — "[X] ago" → always past simple. "She has presented 3 weeks ago" = WRONG. "She presented 3 weeks ago" = CORRECT.**

R10.11 Medical records: "Mr Jones' medical records reveal/show that he has [condition]." Use present simple for reveal/show.

R10.12 Past perfect (had + past participle): AVOID in OET. Alternative: ING form or noun. "She reported non-compliance with the advice given."

R10.13 Reported speech — after reported/mentioned/stated/described: use ING or noun. "She reported having headaches." NOT "...that she had had headaches."

R10.14 "Next visit" in past narrative → "on the following visit." Never use "next."

11 MEDICATIONS & INVESTIGATIONS

R11.1 **CRITICAL — Latin abbreviations MUST be translated: od/om → once a day; bd/bid → twice a day; tds/tid → three times a day; qds/qid → four times a day; stat → immediately; prn → as needed.**

R11.2 Single medication: Drug, dose. Example: "Panadol, 500 mg."

R11.3 Multiple medications: Drug, dose; Drug, dose; and Drug, dose. (comma: drug-dose; semicolon: between pairs; "and" before last drug.)

R11.4 Trade name: Capitalise (Panadol, Augmentin). Generic: lowercase (paracetamol, amoxicillin). Use one type consistently.

R11.5 Routes: "oral" in full (never "PO"); IM, IV, SC, SL abbreviations acceptable. Route before drug: "oral Panadol, 500 mg."

R11.6 Elevated values: "an elevation in glucose" / "an increased glucose level" / "elevated glucose." Reduced: "a reduction in glucose" / "a decreased glucose level." Do not write "a glucose" alone.

R11.7 "Glucose level" can take article: "an increased glucose level." Plural modifier: no article: "increased white cell counts."

R11.8 **CRITICAL — Always include numerical values with units when given. Example: "glucose of 7.8 mmol/L." Never write the reference range.**

R11.9 "an MRI" and "an X-ray" — both take "an" (initial vowel sound from letter name).

R11.10 Numerals with units always: "38.5°C", "180/110 mmHg", "500 mg."

R11.11 Clinic scheduling fractions: 1/4 = once a week; 1/12 = once a year; 3/52 = three times a year; 3/7 = three times a week; 8/24 = eight-hourly; 12/24 = twice a day.

12 GRAMMAR, VOCABULARY & LINKERS

R12.1 **CRITICAL — NO contractions: "do not" not "don't"; "I am" not "I'm"; "cannot" not "can't".**

R12.2 **CRITICAL — Never use the word "patient" in the body. Use last name + title, or a pronoun.**

R12.3 Never use "smoker/drinker" as labels. Write "she smokes" / "she drinks alcohol."

R12.4 Replace "tired" with "fatigue." Replace "bad" with "poor" or "inadequate." No colloquial language.

R12.5 **CRITICAL — Medical conditions: LOWERCASE. bronchial asthma, myocardial infarction, hypertension, gastroenteritis. Capitalising these is an error.**

R12.6 Named (eponymous) diseases: CAPITALS. Parkinson's, Crohn's, Alzheimer's, Graves', Doppler.

R12.7 Trade medication names: capitalise. Bacteria: first word capital, second lowercase (Staphylococcus aureus). Abbreviations: all capitals (MRI, CT, ECG, A&E;).

R12.8 Numbers 1–9 in running text: write as words. Numbers 10+: numerals. Exception: always numerals with units ("500 mg").

R12.9 **CRITICAL — "However": always preceded by semicolon; comma follows. Pattern: "[clause]; however, [clause]."**

R12.10 "Therefore" / "Thus": preceded by semicolon; comma follows. Pattern: "[clause]; therefore, [clause]."

R12.11 "In addition" (joining two complete clauses): semicolon before + comma after. "[Clause]; in addition, [clause]."

R12.12 "In addition to" / "together with" / "along with" / "as well as": NO preceding punctuation; followed by noun or ING. "In addition to vomiting, she reported nausea."

R12.13 Proximity rule (negative sentence with "or"): verb agrees with CLOSEST subject. "No orthopnoea or ankle swelling was reported." (swelling = singular → was).

R12.14 Reverse rule ("along with" / "together with" / "in addition to"): verb agrees with word BEFORE the linker. "The rash, along with the fever, was noted." (rash = singular → was).

R12.15 "Despite" + noun/ING (not a full clause). "Although" + full clause. Both acceptable.

R12.16 "As" (meaning because) in mid-sentence: comma before. "Treatment was changed, as the patient showed no improvement."

R12.17 "For which": comma before. "She has hypertension, for which she was commenced on amlodipine."

R12.18 Coordinating adjectives (same noun): comma between them. "He had a severe, persistent headache."

R12.19 Sentence length: 15–25 words target. Do not chain 3+ clauses. No comma splices.

R12.20 Max 1–2 linkers per paragraph. Excessive linkers reduce Conciseness and Clarity score.

R12.21 Appositive: between two commas. "Ms Miller, a 25-year-old teacher, presented with..." Use "gentleman" / "lady" (not man/woman).

R12.22 Never label patients: "anxious", "non-compliant." Write: "she expressed concern about..." / "he reported difficulty adhering to..."

13 URGENT REFERRAL

R13.1 Three elements change in an urgent referral: (1) Introduction contains "urgent", (2) Closure contains "at your earliest convenience", (3) Body starts with today's visit.

R13.2 **CRITICAL — Introduction MUST contain "urgent": "I am writing to urgently refer..." / "I am writing to request urgent assessment of..."**

R13.3 **CRITICAL — Closure MUST include "at your earliest convenience." Spelling: C-O-N-V-E-N-I-E-N-C-E. Do not repeat "urgent" after the introduction.**

R13.4 **CRITICAL — Body: ALWAYS start with TODAY'S VISIT (acute presentation), then background chronologically. Exception: minimal info today → start with preceding visit.**

R13.5	Structure: P1 = today's emergency. P2+ = background history. Final paragraph = active medical conditions + current medications.
R13.6	CRITICAL — Any ongoing medical condition with active medications MUST be mentioned, even if unrelated to the urgent issue.
R13.7	Fully resolved conditions with no current medications: do NOT include.
R13.8	Penicillin allergy: include only if patient is receiving or needs antibiotics.
R13.9	No past medical history? Simply omit that paragraph.
R13.10	CRITICAL — "ASAP" in case notes → write "at your earliest convenience" in the letter. Never write "ASAP."
R13.11	A&E; and ED: always capitalised (A&E;, ED, Emergency Department).

14 DISCHARGE LETTER

- R14.1** Written by hospital doctor to GP; updates GP on admission and discharge.
- R14.2** **CRITICAL — Introduction: "I am writing to update you regarding Mr Jones, who underwent [procedure] at [hospital] and is now ready for discharge, for whom I am requesting your ongoing care." NEVER use routine referral language.**
- R14.3** **CRITICAL — Do NOT describe patient identity in introduction — GP knows the patient.**
- R14.4** **CRITICAL — Do NOT include: family history, social history, smoking/drinking, previous medical history, occupation. GP knows all of this.**
- R14.5** Exception: mention allergy only if it directly influenced treatment. "Due to the documented penicillin allergy, [alternative] was prescribed."
- R14.6** **CRITICAL — Admission: "Mr Jones was admitted to [hospital] with [symptoms/condition]." Past simple. NEVER "was presented" (incorrect passive).**
- R14.7** **CRITICAL — Discharge plan paragraph = most important. Must include: discharge medications with doses + post-discharge instructions.**
- R14.8** Discharge medications: same format — Drug, dose; Drug, dose and Drug, dose.
- R14.9** **CRITICAL — All investigations performed during admission (including normal results) MUST be listed with values.**
- R14.10** **CRITICAL — Follow-up / review dates: ALWAYS include if stated in case notes.**
- R14.11** Consent: "Please note that Mr Jones has been fully informed of his diagnosis and management."
- R14.12** **CRITICAL — "Treatment FOR" not "Treatment FROM." Always: treated for, admitted for, referred for, managed for.**
- R14.13** Past tense throughout (all events occurred during admission).
- R14.14** Copy of results stated as enclosed → "Please find enclosed a copy of Mr [Name]'s pathology results." Do not list values.

15 REFERRAL TO NON-MEDICAL PROFESSIONALS

- R15.1** Non-medical professionals: OT, physiotherapist, social worker, psychologist, dietitian, speech pathologist, pharmacist, care facility manager. Nurse is medical.
- R15.2** **CRITICAL — Do NOT use medical jargon. Translate: "hypertension" → "high blood pressure"; avoid abbreviations like "BP" unless previously spelled out.**
- R15.3** Salutation: Dear Sir/Madam (no name). Dear Mr/Ms [Last Name] (name given).
- R15.4** Include: condition in plain language, functional impact, specific professional request (OT: home assessment; physio: mobility/rehab), relevant social context.
- R15.5** Exclude: medical details irrelevant to the professional's scope.
- R15.6** Family dentist: does NOT know the patient — treat like a specialist; include relevant background.
- R15.7** **CRITICAL — Imperative items in the plan = for GP. Items on the referral request line = for the specialist. Never ask the specialist to perform GP tasks.**
- R15.8** Mental health referrals: correlate symptoms across visits; include review date; include consent statement if mentioned; include "upon his/her request" if patient initiated.

16 ASSESSMENT CRITERIA

- R16.1** Six marking criteria: (1) Purpose — max 3 marks; (2) Content — max 7; (3) Conciseness & Clarity — max 7; (4) Genre & Style — max 7; (5) Organisation & Layout — max 7; (6) Language — max 7.
- R16.2** **CRITICAL — Purpose is the ONLY criterion scored 0–3. All others are scored 0–7.**

R16.3	CRITICAL — Most heavily weighted for pass/fail: Content and Conciseness & Clarity.
R16.4	Genre and Style has significant impact on the overall result.
R16.5	CRITICAL — Language does not need to be perfect. Most OET failures result from poor content selection, not grammar errors.
R16.6	Required score: approximately Band 6+ in most criteria; at least Band 2 out of 3 in Purpose.
R16.7	CRITICAL — Examiner standard: "Could this letter be sent in a real clinical setting?" Yes = pass. No = fail.
R16.8	Assessment is holistic. Do not attempt to calculate an exact score.

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